

PROSPECT PROFILE

Midwest Regional Health System · 2400 N. Lakeshore Drive, Chicago, IL 60657

SYSTEM SIZE 1,200 beds - Multi-site ambulatory + inpatient	PAYER MIX - Heavy Medicaid - Value-based care contracts - Commercial supplement	CBO PARTNERS - Chicago Maternal Health Collective - Chicago Food Pantry Network
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SDOH PRIORITY	IDENTIFIED VIA
Maternal Health Outcomes	CHNA
Food Insecurity	CHNA, Community Program Initiative
Transportation Barriers	CHNA, Quality Improvement Initiative
Preventable ER Utilization	Quality Improvement Initiative

TECH SCOPE

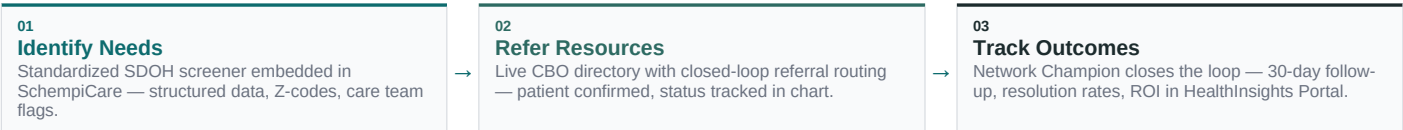
EHR	SchempiCare
Security Assessment	Not completed — initial demo scheduled Thursday, May 7, 2026

PRESENTATION ATTENDEES

STAKEHOLDER	WHAT THEY CARE ABOUT	PAIN POINTS	WIN CONDITION
Steve Gryglas CMIO Primary Buyer Executive Stakeholder	- Clinical workflow fit - SchempiCare integration <3 min workflow to drive provider adoption - Patient safety	- Referrals vanish after documentation - Providers won't adopt tools that add friction	<3 min workflow - Native SchempiCare panel - Screener delegates pre-visit
Erin Kreider VP, Population Health Executive Stakeholder	- Reduce readmissions tied to unmet social needs - VBC contract performance - Chronic condition management	- No visibility into whether SDOH referrals improve outcomes - Can't attribute interventions to claims data	- Measurable outcome data tied to claims - SDOH dashboard linked to VBC metrics
Christy Garuccio VP, Clinical Informatics IT Lead Executive Stakeholder	- SchempiCare integration architecture - Usability & scalability - Implementation timeline	- Free-text SDOH data unusable for reporting - Prior integrations created IT debt - No structured Z-code capture	- SMART on FHIR - Structured Z-codes write to chart - Phased rollout, low IT lift
Katherine Golarz SDOH Program Director Clinical Champion	- Staff adoption - CBO coordination - Change management - Reduce manual burden on care teams	- Providers overwhelmed by SDOH workflow - Manual CBO tracking via spreadsheet - No confirmation referrals are acted on	- Platform reduces manual work - Surfaces only when relevant - No added clicks for providers

PRODUCT OVERVIEW

Most health systems can identify social needs. Few can confirm they were resolved. HealthLink Compass closes that gap — extending SchempiCare to coordinate CBO referrals, track outcomes, and give care teams real visibility into whether interventions actually reached patients. This becomes especially important as health systems shift toward value-based care, where social factors directly impact outcomes and reimbursement.



PRODUCT FIT MATRIX

THE CHALLENGE	HEALTHLINK COMPASS ADDRESSES IT BY...	VALUE ADD · STAKEHOLDER
Referrals sent with no confirmation of receipt or delivery	Network Champions embedded in CBOs confirm intake and close the loop — removing manual follow-up for care teams and updating status directly in SchempiCare	Clinical: Care team knows the referral landed · Operational: Eliminates SW manual tracking → Gryglas, Golarz
SDOH assessments inconsistent — paper in ED, limited SchempiCare module outpatient only	Standardized screener (3 modes: in-visit, device, patient-initiated) writes structured Z-codes to SchempiCare — replacing paper and eliminating dual documentation	Clinical: Consistent, structured SDOH data across all sites · Operational: Single workflow → Gryglas, Garuccio
No outcome measurement — can't report ROI or demonstrate VBC performance	30-day follow-up surveys + SchempiCare claims data generate resolution rates and outcome scores — producing board-ready numbers tied to VBC contracts	Financial: Quantifiable ROI for payers and board · Operational: VBC reporting enabled → Garuccio, Kreider
No population-level visibility into screening rates or provider activity	HealthInsights Portal delivers real-time screening rates, referral volume, resolution rates, and provider activity — all in one dashboard without leaving SchempiCare	Operational: System-level QI accountability · Financial: Community benefit documentation → Kreider, Garuccio

HOW WE COMPARE

Prep Note: Midwest Regional uses paper-based SDOH assessments in the ED and SchempiCare's native SDOH module for outpatient visits only. No closed-loop referral tracking exists in either workflow.

CAPABILITY	HealthLink Compass	Unite Us	Findhelp	SchempiCare Native
Closed-loop referral confirmation	✓	✓	Limited	✗
Native EHR embed (SMART on FHIR)	✓	Limited	Limited	✓
Embedded CBO manager (Network Champion)	✓	✗	✗	✗
Patient-reported outcome surveys	✓	Limited	✗	✗
Provider ↔ CBO in-platform messaging	✓	Limited	✗	✗
Population-level outcomes dashboard	✓	✓	Limited	Limited
Under 3-min clinical workflow	✓	✗	✗	✓

Market context: Unite Us acquired NowPow (2021). Findhelp is the rebrand of Aunt Bertha (2021). Two scaled competitors remain: Unite Us (closed-loop, large health systems) and Findhelp (#1 KLAS-rated SDoH network). Neither embeds a human resolution layer at the CBO — HealthLink Compass's sole differentiator.

DEMO PLAN

Demo Presented By	Sunan Fatima, Clinical Sales Engineer
Demo Site	Custom demo environment built for Midwest Regional Health System
Demo Objective	Demonstrate how HealthLink Compass closes the SDOH loop — from structured assessment to confirmed resolution — with measurable outcomes tied to Midwest Regional's priorities.

DEMO FLOW

#	STEP	WHAT TO SHOW
1	SDOH Assessment	Embedded SchempiCare screener — 3 modes (in-visit, device, patient-initiated). Z-code write-back to chart.
2	Refer to Resources	From identified need, navigate CBO directory. Filter by fee, proximity, language. Send closed-loop referral to Chicago Food Pantry Network.
3	Closed-Loop Workflow	Referrals tab: sent → accepted → delivered → closed in SchempiCare. Provider ↔ CBO Network Champion messaging inside chart.
4	Data & Dashboards	HealthInsights Portal: org-level screening rate, referral volume, resolution rate, 30-day outcomes. Network view: CBO performance by partner. Tie to CHNA priorities.